

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase Ib/II Open-label, Multi-center Study of AMO959 With Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) in Combination With an Androgen Receptor Pathway Inhibitor (ARPI) in Adult Participants With PSMA-positive Metastatic Castration Resistant Prostate Cancer (mCRPC)
Short Title:	A Phase Ib/II Open-label Study of AMO959 With Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) in Combination With ARPI in Adult Participants With PSMA-positive mCRPC
Protocol Number:	1079774794003581831
NCT Number:	NCT07226986
Sponsor Name:	Novartis
Investigational Product:	abiraterone
Phase:	PHASE1, PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Phase Ib: Incidence rate of Dose-limiting toxicities (DLTs): Incidence of dose limiting toxicities (DLTs) with AMO959 in combination with AAA617 +/- ARPI A dose-limiting toxicity (DLT) is defined as an adverse event or abnormal laboratory value assessed as unrelated to disease, disease progression, inter-current illness/injury, or concomitant medications that occurs within the evaluation period and meets any of the criteria specified in the protocol. The National Cancer Institute Common Terminology Criteria for Adverse events (NCI CTCAE) version 5.0 will be used for all grading. 2. Phase Ib: Incidence rate of Adverse Events (AEs) and Serious Adverse Events (SAEs): Incidence of adverse events by type, frequency, and severity, as graded by the NCI CTCAE version 5.0. 3. Phase Ib: Number of Participants with dose adjustments: The number of participants with dose adjustments (reductions, interruption, or permanent discontinuation).
Secondary Obj.:	1. Prostate Specific Antigen 90 (PSA90) response 2. Phase Ib: Prostate Specific Antigen 50 (PSA50) response 3. Phase Ib and Phase II: Radiographic progression-free survival (rPFS)

2.2 Background & Rationale

The purpose of this phase Ib/II study is to (a) in Phase Ib evaluate the safety, tolerability, and pharmacokinetics (PK) of AMO959 when given in combination with lutetium (177Lu) vipivotide tetraxetan (also known as $[^{177}\text{Lu}]\text{Lu-PSMA-617}$ or 177Lu-PSMA-617 and hereafter referred to as AAA617) with an androgen receptor pathway inhibitor (ARPI) in participants with metastatic castration resistant prostate cancer (mCRPC) who have failed one prior ARPI and with or without prior taxane exposure, and (b) in Phase II evaluate the preliminary efficacy of AMO959 in combination with

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

AAA617 and ARPI in participants with mCRPC who have failed one prior ARPI, but who have not yet been exposed to taxane treatment.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL,	Phase: PHASE1, PHASE2,	Status:
	NOT_YET_RECRUITING		
Target N:	123		

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Signed informed consent must be obtained prior to participation in the study.
- * Participants must be adults ? 18 years of age.
- * Participants must have an ECOG performance status of 0 to 2.
- * Participants must have histologically confirmed adenocarcinoma of the prostate. Participants with other histology (e.g. neuroendocrine, intraductal subtype) are not eligible.
- * Phase Ib: Prior exposure of up to 1 line of taxane-based chemotherapy is permissible. Phase II: Participants must not have received taxane-based chemotherapy in mCRPC setting (allowed in mHSPC setting).
- * Participants must have PSMA-PET positive disease assessed by using a PSMA imaging agent that is approved as per protocol and are eligible as determined by the sponsor's central reading rules.
- * Castration level of testosterone (< 50 ng/dL), and/or use of concomitant ADT
- * Participant must have been diagnosed with mCRPC with documented progressive disease while on treatment with ARPI in mHSPC or earlier setting as their last treatment (and did not progress on more than one ARPI), based on at least 1 of the following criteria:
 - * Serum/plasma PSA progression is defined as 2 increases in PSA measured at least 1 week apart. The minimal start value is 2.0 ng/mL; 1.0 ng/mL is the minimal starting value if confirmed rise in PSA is the only indication of progression as per PCWG3 guidelines.
 - * Soft-tissue progression defined PCWG3-modified RECIST v1.1 (Eisenhauer et al 2009, Scher et al 2016).
 - * Progression of bone disease: 2 new lesions; only positivity on the bone scan defines metastatic disease to bone (PCWG3 criteria Scher et al 2016).

Key Exclusion Criteria:

- * Concurrent local (radiation therapy to the prostate with curative intent or other prostate antineoplastic ablative procedures) or systemic (hormonal ablation, chemotherapy, immunotherapy, , RLTs) antineoplastic treatments, or within 28 days of enrollment (Phase Ib) or randomization (Phase II)
- * Prior treatment with any RLT or PSMA-targeted agents (approved or investigational)
- * Any other investigational agents within 28 days prior to first dose of any study treatment
- * Concurrent serious medical conditions that may interfere with study procedures or followup
- * Participants with a history of CNS metastases must have received therapy (surgery, whole brain radiation therapy,

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

stereotactic radiosurgery) and be neurologically stable, asymptomatic, and not taking corticosteroids for the purpose of maintaining neurologic integrity.

Other protocol-defined inclusion/exclusion criteria may apply.

4.2 Exclusion Criteria

- * Concurrent local (radiation therapy to the prostate with curative intent or other prostate antineoplastic ablative procedures) or systemic (hormonal ablation, chemotherapy, immunotherapy, , RLTs) antineoplastic treatments, or within 28 days of enrollment (Phase Ib) or randomization (Phase II)
- * Prior treatment with any RLT or PSMA-targeted agents (approved or investigational)
- * Any other investigational agents within 28 days prior to first dose of any study treatment
- * Concurrent serious medical conditions that may interfere with study procedures or followup
- * Participants with a history of CNS metastases must have received therapy (surgery, whole brain radiation therapy, stereotactic radiosurgery) and be neurologically stable, asymptomatic, and not taking corticosteroids for the purpose of maintaining neurologic integrity.

Other protocol-defined inclusion/exclusion criteria may apply.

11. Study Identifiers & Governance

Primary ID:	1079774794003581831
Registry Number:	NCT07226986
Sponsor:	Novartis
Study Title:	A Phase Ib/II Open-label Study of AMO959 With Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) in Combination With ARPI in Adult Participants With PSMA-positive mCRPC

15. Sign-off & Approval

Principal Investigator: _____	Date: _____
Clinical Operations Lead: _____	Date: _____
Lead Statistician: _____	Date: _____